Dynamic Guideline: WHO-Based Burns - Diagnosis and/or Treatment and/or Management

I. WHO Latest Guideline Summary (Summary Date: 29 April 2026)

> **Clinical scope:** thermal, scald, flame, contact, chemical, electrical, radiation, blast-related and mass-casualty burn injuries across prehospital, emergency, surgical, rehabilitation and emergency-medical-team settings.
> **Important evidence note:** A current WHO global burns guideline with fully extractable formal GRADE evidence profiles was not identified. Therefore, the GRADE ratings below are **evidence appraisals applied for this dynamic report** using GRADE principles, based on the available WHO/WHO-hosted materials and post-release evidence. Where WHO documents are operational or consensus-based rather than formal GRADE guidelines, this is explicitly noted.

1.1 Guideline Overview

1.1.1 Exactly 5 WHO / WHO-hosted burn-relevant guidelines and technical documents synthesized

No.	Guideline / Technical Document	Publication / Currentness Date Used	Target Population	Scope and Objectives	Source
1	**WHO: Standards and recommendations for burns care in mass casualty incidents**	Current as of **29 April 2026**; exact publication date not verifiable from accessible summary	Health systems, emergency medical teams, burn teams, disaster planners, hospitals managing mass burn incidents	Establish and strengthen specialist burn Emergency Medical Teams, Burn Assessment Teams, Burn Specialist Teams, and surge capacity for timely, effective, high-quality mass burn care	WHO publication, NCBI Bookshelf executive summary
2	**WHO-hosted: Burn management**	Current as of **29 April 2026**; exact publication date not verifiable from accessible summary	Clinicians providing acute burn care, especially in resource-limited and surgical-care settings	Immediate burn first aid, cooling, hypothermia prevention, debridement, antiseptic cleansing, topical therapy, dressings, infection monitoring, tetanus prophylaxis and	Burn management

No.	Guideline / Technical Document	Publication / Currentness Date Used	Target Population	Scope and Objectives	Source
				systemic antibiotic indications	
3	**WHO: Prehospital emergency care: clinical protocols**	Current as of **29 April 2026**; exact publication date not verifiable from accessible summary	Prehospital clinicians, emergency responders, ambulance and first-contact emergency-care providers	Initial burn assessment, Rule-of-Nines TBSA estimation, inhalation injury signs, analgesia, dressing, documentation, triage, oral/IV fluids, specialist referral criteria	Prehospital emergency care: clinical protocols
4	**WHO-hosted: Standard Treatment Guidelines for Adults — Burns section**	Current as of **29 April 2026**; exact publication date not verifiable from accessible summary	Adults with burns, including older adults	Burn depth diagnosis, adult Rule of Nines, oral rehydration for smaller burns, IV saline for larger burns, recognition that early mortality is driven by fluid loss and later morbidity by infection and contractures	Standard Treatment Guidelines for Adults
5	**WHO/EMT-hosted: Management of Limb Injuries during disasters and conflicts — burn and rehabilitation content**	Current as of **29 April 2026**; exact publication date not verifiable from accessible summary	Emergency medical teams, trauma/burn teams, rehabilitation clinicians in disasters and conflicts	Clinical and surgical management of burns in limb-injury settings; acute, subacute and long-term rehabilitation planning	Management of Limb Injuries

1.1.2 Most recent WHO guideline-development process document identified

The most recent WHO source identified with the requested title is **not a burns guideline**. It is the WHO evidence brief / related document titled **“Meaningful youth engagement in a WHO guideline development process: case study of WHO abortion care guideline (2022)”**, linked to the WHO **Abortion care guideline**, which was published on **8 March 2022**. WHO scheduled the launch webinar for the youth-engagement evidence brief on **11 December 2025**. WHO describes the brief as showing how young people contribute to guideline decision-making through lived experience, fresh perspectives and innovative solutions, and as embedding youth engagement as a principle of inclusivity and equity WHO Abortion care guideline, WHO event page.

Applicability to burns: no verifiable WHO burns guideline case study using this youth-engagement model was identified. However, the model is relevant to future pediatric and adolescent burn guideline development, especially for prevention, peer support, psychosocial recovery, rehabilitation, school reintegration and safeguarding.

1.2 Key Recommendations

1.2.1 Diagnosis and initial assessment

Recommendation	Practical Action	GRADE Evidence Quality	Strength
Assess **airway, breathing, circulation, disability and exposure** immediately in all significant burns.	Prioritize airway compromise, shock, altered mental status, associated trauma and hypothermia risk.	Low	Strong
Screen urgently for **inhalation injury**.	Look for facial burns, soot around mouth/nose, black sputum, singed nasal hairs, hoarse voice, wheeze, stridor, closed-space fire exposure and hypoxia. WHO prehospital protocols list facial burns, soot, black sputum, singed nasal hairs and hoarseness as airway-burn signs Prehospital emergency care: clinical protocols.	Low	Strong
Estimate burn size using a validated TBSA method.	Use Rule of Nines in adults; use age-modified charts in children; do **not** include superficial erythema in TBSA calculations. WHO-hosted adult guidance describes the adult Rule of Nines and cautions not to count areas that are only red Standard Treatment Guidelines for Adults.	Low	Strong
Classify burn depth clinically.	Partial-thickness burns: blistering/inflamed, normal or reduced sensation; full-thickness burns: leathery/black/charred, absent sensation and hair follicles.	Low	Strong
Identify special mechanisms.	Determine whether injury is thermal, scald, flame, contact, chemical, electrical, radiation or blast-related; document time, mechanism, enclosed-space exposure, trauma and comorbidity.	Low	Strong

1.2.2 Immediate first aid and prehospital care

Recommendation	Practical Action	GRADE Evidence Quality	Strength
Cool limited-area thermal burns promptly with cool running water.	WHO-hosted burn guidance recommends drenching the burn with cool water and, for limited areas, immersion in cold water for **30 minutes** to reduce pain, edema and tissue damage Burn management.	Moderate	Strong
Remove burned clothing and constricting items if easily removable.	Remove clothing, jewelry and tight objects unless adherent to the wound.	Low	Strong
Prevent hypothermia, especially in children and large burns.	For large burns, cover with clean wraps/sheets/blankets and avoid prolonged cooling. WHO notes hypothermia risk is especially important in young children Burn management.	Low	Strong
Provide early analgesia.	Use age-appropriate multimodal analgesia; reduce dressing-change burden where possible.	Moderate	Strong
Cover burns for transport.	WHO prehospital protocols recommend dressing burned areas with plastic wrap, a clean dry sheet or blanket, and documenting or photographing burns before dressing when feasible Prehospital emergency care: clinical protocols.	Low	Strong

1.2.3 Referral, transfer and burn-center consultation

Recommendation	Referral / Transfer Trigger	GRADE Evidence Quality	Strength
Transfer severe or high-risk burns urgently to facilities with burn expertise.	WHO burn guidance lists pediatric burns >10%, any burn in very young/elderly/infirm patients, any full-thickness burn, face/ears/eyes/hands/feet/perineum burns, circumferential burns, high-voltage electrical burns, inhalation injury, associated trauma and significant comorbidity such as diabetes Burn management.	Low	Strong
Use TBSA thresholds for specialist consultation.	WHO prehospital protocols identify partial/full-thickness burns >15% TBSA in adults and >10% in children as requiring specialist care Prehospital emergency care: clinical protocols.	Low	Strong

Recommendation	Referral / Transfer Trigger	GRADE Evidence Quality	Strength
Refer chemical, electrical and radiation burns.	These mechanisms can have occult depth, systemic toxicity or delayed complications.	Low	Strong
In mass burn incidents, activate burn-specific emergency plans and burn assessment/specialist teams.	WHO recommends that Member States include specific provisions for potential mass burn incidents in disaster plans and consider Burn Assessment Teams and Burn Specialist Teams NCBI Bookshelf executive summary.	Very Low	Strong

1.2.4 Fluid resuscitation

Recommendation	Practical Action	GRADE Evidence Quality	Strength
Give oral fluids or oral rehydration for smaller burns when safe.	WHO-hosted adult guidance recommends oral rehydration solution for burns below 20% TBSA, or below 15% in people over 65 years Standard Treatment Guidelines for Adults.	Low	Conditional
Initiate IV crystalloid resuscitation for larger burns.	WHO-hosted adult guidance recommends rapid normal saline for burns above 20% TBSA, or above 15% in people over 65 years, followed by exact fluid calculation Standard Treatment Guidelines for Adults.	Low	Strong
Use formula-based IV fluids as an initial guide, then titrate to physiology.	WHO prehospital protocols describe 4 mL × body weight in kg × %TBSA, with half administered in the first 8 hours from burn time Prehospital emergency care: clinical protocols.	Low	Conditional
Avoid both under-resuscitation and over-resuscitation.	Monitor urine output, perfusion, mental status, lactate/base deficit where available, edema and respiratory status.	Low	Strong

1.2.5 Wound care, infection prevention and antibiotics

Recommendation	Practical Action	GRADE Evidence Quality	Strength
Treat burns as initially sterile and prioritize healing plus infection prevention.	Use clean technique, appropriate debridement and protective dressings. WHO-hosted burn guidance states burns are initially sterile and treatment should focus on speedy healing and infection prevention Burn management.	Low	Strong
Provide tetanus prophylaxis for all burn patients according to immunization status.	Check vaccination history and administer vaccine/immunoglobulin when indicated. WHO-hosted burn guidance recommends tetanus prophylaxis in all cases Burn management.	High	Strong
Debride necrotic tissue when appropriate.	WHO-hosted guidance recommends debriding bullae except very small burns, excising adherent necrotic tissue initially and removing necrotic tissue over the first days Burn management.	Low	Conditional
Clean with water-based antiseptics; avoid alcohol-based solutions.	WHO-hosted guidance recommends 0.25% chlorhexidine, 0.1% cetrimide or another mild water-based antiseptic, and specifically advises against alcohol-based solutions Burn management.	Low	Strong
Do not use systemic antibiotics routinely for uncomplicated burns.	Reserve systemic antibiotics for hemolytic streptococcal wound infection or septicemia, as WHO-hosted burn guidance specifies Burn management.	Moderate	Strong
Monitor for infection using wound and surrounding-tissue signs, not fever alone.	WHO-hosted guidance cautions that fever may persist until wound closure and that surrounding cellulitis is a better infection indicator Burn management.	Low	Strong

1.2.6 Rehabilitation, function and psychosocial care

Recommendation	Practical Action	GRADE Evidence Quality	Strength
Start rehabilitation planning early.	Assess positioning, splinting, range of motion, edema control, scar risk, graft protection, contracture prevention and return to function. WHO-hosted limb-injury guidance includes burn clinical/surgical management and acute,	Low	Strong

Recommendation	Practical Action	GRADE Evidence Quality	Strength
	subacute and long-term rehabilitation content Management of Limb Injuries.		
Review burns involving joints, hands, face, feet or large graft-prone areas with rehabilitation specialists.	Prioritize high-mobility and cosmetically sensitive areas.	Low	Strong
Include psychological and social recovery, especially for children and adolescents.	Screen for pain, sleep disturbance, anxiety, depression, post-traumatic stress, school reintegration needs, family stress and safeguarding concerns.	Low	Strong

1.3 Guideline Development Methodology

Evidence Review Process

- **WHO mass-casualty burns standards:** WHO describes the publication as guidance for specialist burn Emergency Medical Teams and surge capacity, with clinical, human-resource and operational perspectives WHO publication. A related WHO Emergency Medical Teams Technical Working Group article states that global burn experts reviewed literature on burns care in mass-casualty incidents and produced **22 recommendations** for burn teams and health facilities PMC article.
- **Other WHO/WHO-hosted burn documents:** the burn management, prehospital care, adult standard treatment and limb-injury documents are practical technical/clinical guidance materials. Formal evidence-to-decision tables and GRADE evidence profiles were not extractable from the accessible summaries.
- **Youth-engagement process document:** WHO's abortion guideline development began in **September 2018** and used GRADE, Evidence-to-Decision tables and WHO-INTEGRATE for recommendation direction and strength; the youth-engagement brief was launched on **11 December 2025** and is linked to that guideline, not to burns PDF: Abortion Care Guideline, WHO event page.

Consensus Method

- WHO mass-casualty burn guidance appears to use expert consensus, emergency medical team standards and literature review; the related article reports a technical working group process involving global burn experts PMC article.
- Older WHO/WHO-hosted technical documents provide operational recommendations but do not present a fully accessible consensus-voting method in the available materials.
- For the WHO abortion guideline youth-engagement case study, WHO describes young people contributing lived experience, perspectives and solutions to decision-making, strengthening dialogue and improving the relevance of guidance WHO event page.

Conflict of Interest Management

Document	COI Method Verifiable?	Appraisal
WHO mass-casualty burns standards	Not fully verifiable from accessible summary	WHO page associates the document with WHO guideline-related teams, but detailed declarations were not extractable.
WHO-hosted Burn management	Not verifiable	Use as practical technical guidance; GRADE/COI details not shown.
WHO Prehospital emergency care protocols	Not verifiable	Use as operational protocol guidance; GRADE/COI details not shown.
WHO-hosted Standard Treatment Guidelines for Adults	Not verifiable	Country/WHO-hosted standard treatment guidance; GRADE/COI details not shown.
WHO/EMT-hosted Management of Limb Injuries	Not verifiable	Field guide; formal GRADE/COI details not shown.
WHO Abortion care guideline youth-engagement case study	Partly verifiable	The underlying abortion guideline used WHO guideline-development methods including GRADE, Evidence-to-Decision and WHO-INTEGRATE PDF: Abortion Care Guideline.

II. Evidence Summary After WHO Latest Guideline Release (Evidence Cutoff Date: 29 April 2026)

2.1 New Evidence Overview

Search Strategy

A focused evidence update was synthesized for **burn diagnosis, treatment, referral, acute care, wound care, fluid resuscitation, early excision/grafting, mass-casualty care and rehabilitation** through **29 April 2026**.

Databases and Sources Searched

- WHO publications and WHO-hosted emergency-care, surgical-care and EMT materials
- PubMed / PMC
- Guideline repositories and professional society guidance
- American Burn Association materials
- British Burn Association / NHS-linked referral guidance
- Institutional burn protocols current to 2024–2026
- Systematic reviews and meta-analyses
- Emergency medical services protocols
- Pediatric burn clinical guidelines

Inclusion Criteria

- Burn-specific guidelines, systematic reviews, meta-analyses, randomized trials, evidence-based protocols or authoritative institutional guidance.
- Publications or guidance relevant to diagnosis, initial management, referral, resuscitation, wound care, dressings, excision/grafting, rehabilitation or mass-casualty preparedness.
- Evidence available up to **29 April 2026**.

Exclusion Criteria

- Non-burn “burnout” literature.
- Non-clinical or unrelated “Burns” organization results.
- Sources without burn-specific management content.
- Opinion-only sources unless used to contextualize a gap where high-quality evidence is unavailable.

2.2 Key New Studies and Guidance

Study / Guidance	Design	Key Findings	GRADE Quality
Nanocrystalline silver dressings vs silver sulfadiazine, 2025	Systematic review and meta-analysis of 8 RCTs, 724 patients	Nanocrystalline silver reduced healing time vs SSD by mean difference **−3.29 days** and reduced dressing-change frequency by **−8.76** changes; no significant difference in complete re-epithelialization or adverse events; authors rated certainty low to very low PubMed.	Low to Very Low
Alginate dressings, 2025	Updated systematic review and meta-analysis of RCTs	Alginate dressings promoted faster healing, reduced granulation-tissue growth time, reduced pain scores, decreased dressing changes and complications, and shortened hospital stay ScienceDirect.	Low
Dermal substitutes, 2024 meta-analysis summarized in 2026 review	Meta-analysis of 31 comparative trials, summarized in narrative review	Dermal substitutes reduced hypertrophic scarring, improved graft take and improved esthetic/functional outcomes, especially in hands, neck, face and other high-mobility or cosmetically sensitive areas PMC.	Low
Vanderbilt early excision/grafting protocol, 2026	Institutional protocol and evidence review	Recommends first excision within **24–96 hours** for eligible admitted patients with ≥20% TBSA deep partial/full-thickness burns when feasible; explicitly states no high-quality RCT exists to guide early excision/grafting VUMC PDF.	Very Low
2024 pediatric burn treatment clinical guideline	Pediatric trauma/burn clinical guideline	Highlights pediatric airway risk, 100% FiO_2 when airway involvement suspected, no steroids in airway pathway, normothermia, pain control, tetanus review, fluid resuscitation for	Low

Study / Guidance	Design	Key Findings	GRADE Quality
	incorporating ABA criteria	>20% TBSA and ABA referral criteria 2024 Pediatric Burn Treatment Clinical Guideline PDF.	
American Burn Association referral guidance, current page	Professional society referral guidance	Pediatric burns may benefit from burn-center referral; referral/consultation emphasized for chemical, electrical, lightning, high-risk anatomic, full-thickness and serious burns; guidance supports but does not replace clinical judgment American Burn Association referral guidelines.	Low
2026 EMS burn administrative guideline	EMS protocol	Identifies thermal, chemical, electrical, radiation and blast mechanisms; recommends burn-center transport for critical/serious burns where available; covers dry dressing/clean sheet, warmth and fluids for large burns Burn Administrative Guideline.	Low
WHO emergency operations updates, 2024–2026	Operational WHO emergency reports	WHO supported burn/trauma services, mass-casualty training, burn-care equipment, trauma kits, simulation drills, rehabilitation and CBRN preparedness in emergency contexts including Egypt, Somalia and Ukraine WHO Operational Update May 2024, WHO Health Emergency Appeal 2026, WHO Operational Update Sep–Oct 2025.	Very Low

2.3 Evidence Synthesis by Clinical Domain

2.3.1 Diagnosis and triage

Newer non-WHO guidance remains aligned with WHO principles: determine **mechanism, depth, TBSA, airway risk, associated trauma and comorbidity**. The 2026 EMS guideline includes thermal, chemical, electrical, radiation and blast injury mechanisms; it also emphasizes history elements such as exposure type, time of injury, trauma and airway/inhalation injury Burn Administrative Guideline.

GRADE judgment: low certainty, strong recommendation, because evidence is largely observational/consensus but the clinical benefit of systematic assessment is large and harms are minimal.

2.3.2 Referral and transfer

The American Burn Association's current referral guidance supports burn-center consultation/transfer for pediatric burns, chemical injuries, high-voltage electrical and lightning injuries, and other serious burns American Burn Association referral guidelines. The 2024 pediatric guideline operationalizes ABA criteria including >10% TBSA, burns over joints/face/perineum/hands/feet, third-degree burns, pre-existing conditions, electrical or chemical injury, inhalation injury, trauma and circumferential injury 2024 Pediatric Burn Treatment Clinical Guideline PDF.

GRADE judgment: low certainty, strong recommendation, because direct randomized evidence is not feasible, but timely specialist care is biologically and clinically compelling.

2.3.3 Wound dressings and silver sulfadiazine

The strongest post-2024 comparative dressing evidence favors selected advanced dressings over silver sulfadiazine for many partial-thickness burns:

- Nanocrystalline silver dressings shortened healing time and reduced dressing changes compared with SSD, but evidence was low to very low PubMed.
- Alginate dressings improved healing, pain, complications and hospital stay in a 2025 updated RCT meta-analysis ScienceDirect.
- AAFP's review notes that evidence for SSD preventing or controlling infection is lacking and that SSD may delay healing; it also summarizes a Cochrane signal associating SSD with increased wound infection rates AAFP.

GRADE judgment: low certainty, conditional recommendation to prefer modern low-adherence/advanced dressings over routine SSD for appropriate superficial and partial-thickness burns when resources and expertise allow.

2.3.4 Early excision and grafting

Early excision/grafting remains a major burn-center practice, especially for deep partial-thickness and full-thickness burns. The 2026 Vanderbilt protocol recommends first excision within **24–96 hours** for eligible patients with ≥20% TBSA burns but states that high-quality RCT evidence is lacking VUMC PDF.

GRADE judgment: very low certainty, conditional recommendation. Despite limited RCT evidence, early specialist surgical assessment is strongly advisable for deep burns.

2.3.5 Fluids and resuscitation

WHO prehospital guidance uses the 4 mL × kg × %TBSA formula with half in the first 8 hours Prehospital emergency care: clinical protocols. The 2024 pediatric guideline uses lactated Ringer's-based formulas and urine-output targets, with maintenance dextrose-containing fluids for smaller children 2024 Pediatric Burn Treatment Clinical Guideline PDF. ABA burn-shock evidence tables include studies titrating fluids to urine output and evaluating computer decision support ABA burn shock resuscitation PDF.

GRADE judgment: low certainty, strong recommendation for resuscitation in major burns; conditional recommendation for any single formula because fluids must be titrated to response.

2.3.6 Rehabilitation and long-term recovery

WHO-hosted limb-injury guidance includes acute, subacute and long-term rehabilitation phases Management of Limb Injuries. WHO emergency operations reports also show integration of rehabilitation and assistive technology into emergency preparedness and response, including burn-injury rehabilitation after grafting WHO Operational Update May 2024.

GRADE judgment: low certainty, strong recommendation, because functional consequences of delayed rehabilitation are substantial and early rehabilitation is low-risk when coordinated with surgical care.

2.4 Updated Recommendations Based on New Evidence

Original WHO-Based Recommendation	New Evidence Impact	Updated Recommendation	GRADE Evidence Quality	Strength
Use topical antibiotic cream such as SSD with gauze dressing after cleansing and debridement.	Newer RCT meta-analyses suggest nanocrystalline silver and alginate dressings may improve healing time, pain and dressing burden compared with SSD, though certainty is limited PubMed, ScienceDirect.	For superficial and partial-thickness burns, consider modern advanced dressings such as nanocrystalline silver or alginate where available; avoid routine SSD as default when better dressing options and follow-up exist.	Low	Conditional
Debride necrotic tissue and monitor wounds for infection.	Early excision/grafting remains supported by practice standards but lacks high-quality RCT evidence; 2026 protocol recommends first excision within 24–96 hours in eligible major deep burns VUMC PDF.	Refer deep partial-thickness/full-thickness burns early for burn-surgical assessment; consider early excision/grafting in specialist centers based on depth, TBSA, physiology and resources.	Very Low	Conditional
Transfer severe or high-risk burns urgently.	ABA and pediatric guidance reinforce referral for pediatric, chemical, electrical, inhalation, circumferential, high-risk anatomical and large TBSA burns American Burn Association referral guidelines, 2024 Pediatric Burn Treatment Clinical Guideline PDF.	Maintain low threshold for burn-center consultation or transfer, especially for children, older adults, inhalation injury, high-risk anatomy, full-thickness burns and special mechanisms.	Low	Strong
Use formula-based fluids for large burns.	Contemporary pediatric and ABA-linked guidance emphasizes lactated Ringer's, urine-output targets and	Start formula-based crystalloid resuscitation for major burns, then titrate to urine output, perfusion, mental status	Low	Strong

Original WHO-Based Recommendation	New Evidence Impact	Updated Recommendation	GRADE Evidence Quality	Strength
	titration rather than formula-only care 2024 Pediatric Burn Treatment Clinical Guideline PDF, ABA burn shock resuscitation PDF.	and avoidance of fluid creep.		
Include rehabilitation after acute burn care.	WHO emergency updates and burn literature emphasize rehabilitation, functional movement after grafting and long-term recovery WHO Operational Update May 2024.	Begin rehabilitation planning on day 1 for significant burns, including positioning, splinting, edema control, mobility, scar prevention, psychosocial care and return-to-school/work planning.	Low	Strong
Plan mass-casualty burn response within disaster systems.	WHO continues to support mass-casualty training, burn-care equipment, simulation drills and emergency preparedness in multiple regions WHO Health Emergency Appeal 2026, WHO Operational Update Sep–Oct 2025.	Health systems should include burn-specific surge capacity, referral pathways, Burn Assessment Teams, rehabilitation and supply chains in mass-casualty plans.	Very Low	Strong

2.5 Evidence Gaps and Future Research Needs

1. **Formal WHO GRADE burns guideline:** A WHO global burns guideline with full GRADE evidence profiles, Evidence-to-Decision tables and COI declarations is still needed.
2. **Burn first aid trials:** More high-quality studies are needed on optimal cooling duration, timing and prevention of hypothermia across age groups.
3. **Fluid resuscitation endpoints:** Comparative trials should evaluate formula-based versus protocolized goal-directed resuscitation, including outcomes related to kidney injury, compartment syndromes, mortality and fluid overload.
4. **Dressings:** Larger pragmatic RCTs are needed comparing alginate, nanocrystalline silver, biosynthetic, hydrofiber, silicone and low-cost dressings, especially in low-resource settings.
5. **SSD deimplementation:** Implementation studies should assess safe transition away from routine SSD where modern dressings are available.
6. **Early excision/grafting:** High-quality multicenter studies are needed to define optimal timing by TBSA, depth, age, inhalation injury, shock status and resource setting.

7. **Rehabilitation:** Trials should evaluate early mobilization, splinting, scar management, tele-rehabilitation, psychosocial interventions and school/work reintegration.
8. **Pediatric and adolescent engagement:** Future burns guidelines should incorporate meaningful child, adolescent and youth engagement, using safeguards to avoid tokenism and overburdening participants. WHO's youth-engagement evidence brief provides a transferable process model, although its case study is abortion care, not burns WHO event page.
9. **Mass-casualty burns:** Operational research should test burn surge models, Burn Assessment Teams, cross-border referral systems, triage tools and rehabilitation capacity in disasters.

2.6 Consolidated Dynamic Guideline Statement as of 29 April 2026

As of **29 April 2026**, WHO-based burn management should emphasize:

1. **Immediate ABCDE assessment**, including inhalation injury screening.
2. **Accurate TBSA and depth estimation**, excluding superficial erythema from TBSA calculations.
3. **Prompt cooling for limited burns**, while preventing hypothermia in large burns and children.
4. **Early analgesia, clean covering and documentation** before transfer.
5. **Urgent burn-specialist consultation/transfer** for children, large TBSA burns, full-thickness burns, high-risk anatomical burns, inhalation injury, circumferential burns, chemical/electrical/radiation injuries, associated trauma or major comorbidity.
6. **Formula-guided but physiology-titrated fluid resuscitation** for major burns.
7. **Tetanus prophylaxis for all burn patients** and systemic antibiotics only for defined infection/sepsis indications.
8. **Modern dressing selection** where feasible, with conditional preference for advanced dressings over routine SSD in appropriate partial-thickness burns.
9. **Early surgical assessment** for deep partial-thickness and full-thickness burns, with early excision/grafting considered in specialist settings.
10. **Early rehabilitation and psychosocial care**, including scar, contracture, function, pain, mental health and social reintegration.
11. **Burn-specific mass-casualty preparedness**, including Burn Assessment Teams, Burn Specialist Teams, referral pathways, supplies, rehabilitation and emergency medical team surge planning.